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Consultation and Counseling in Family Medicine

Dr. Saman Ahmed Mohammed
M.B.Ch.B., F.K.B.M.S. / Family Medicine
Lecturer / Branch of Clinical Science



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Objectives

- To understand the concepts of consultation and counseling.
- To understand why are consultation and counseling is important.
- To learn the stages of counseling.
- What are the possible barriers.



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Family Medicine

- ❖ Medical specialty
- ❖ Provide continuing and comprehensive health care for the individual and the family
- ❖ Integrates the biologic, clinical, and behavioral sciences
- ❖ The scope of family medicine encompasses all ages, both sexes, each organ system, and every disease entity.
- ❖ In many countries, the term *general practice* is synonymous with family medicine e.g.



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Principles of Family Medicine

Australia

- Doctor-patient-family relationship
- High quality primary care provider for all age groups
- Community and population-based practice



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Doctor-patient-family relationship

- Effective Communication with patient and family:
Active listening
- Patient-centered medicine
- Counseling / consultation
- Empathy
- Caring than curing
- Difficult patient management



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ROLE

A DESCRIPTION OF THE WORK OF THE GENERAL PRACTITIONER:

- “ The **general practitioner** is a licensed medical graduate who **gives care to individuals** irrespective of age, sex, and illness.

It is the synthesis of these functions which is unique.

- He will attend his patients **in** his consulting room and in their **homes** and sometimes in a clinic or a **hospital**.
- His aim is to make **early diagnosis**.
- He will include and **integrate** physical, psychological and social factors in his considerations about health and illness.



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- **He will** make an **initial decision** about every problem which is presented to him as a doctor.
- **He will** undertake the **continuing management** of his patients with **chronic, current, or terminal illness**.
- **Prolonged** contact means that he can use repeated opportunities **to gather** information at a pace appropriate to each patient and **build up a relationship of trust** which he can use professionally.



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- **He will** practice in **cooperation** with other colleagues, **medical and non-medical.**
- **He will** know how and when to intervene through **treatment, prevention, and education to promote the health of his patients and their families.**
- **He will** recognize that he also has **a professional responsibility to the community.**



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Consultation Skills

- Family Physicians often need to be the bearers of the worst imaginable news.
- They have to arrange complex and uncertain information to something understandable.
- They have to respond to the needs of hugely diverse patients and their families.
- They have to do much of this when they are busy and under pressure.



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What is Counseling?

- It is the skilled and principled use of relationship to help the patient develop self-knowledge, emotional acceptance and growth including personal resources.
- Counselors who offers warmth, genuineness and empathy are more effective.



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Theories of Counseling

- Directive : Active or Directive counselors tend to interpret, lead and direct their clients.
- Non- directive: Non – Directive or Reflective counselors tend to elicit and reflect, guide and support their clients.



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Aims of Counseling

- To help people accept their difficulties and identify ways of coping more effectively and resourcefully.
- The counselor listens and asks questions until both the counselor and the client understand the way the client sees things.
- The counselor enables the client to clarify thoughts and feelings for better understanding of the problem.



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Stages of Counseling

- ❖ Exploration: Let the patient explore the problem and then focus on specific concern.
- ❖ New understanding: to see both themselves and their situation in new perspectives and how to cope more effectively.
- ❖ Goal Setting
- ❖ Action: possible ways to act; cost/ consequences, planning, implementation and evaluations; creative thinking, problem solving and decision making.



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Consultation and Counseling skills

- If a joint understanding of the problem and management plan; which the patient understands, feels comfortable with, and prepared to adhere to; is not made:

((The patient is not likely to follow the advice and all our efforts in assessment and diagnosis are wasted)).

(Silverman et al. 1988)



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Deficiencies in Communication and Counseling

- ☐ Doctors may not obtain enough information regarding patients perspective.
- ☐ Provide information in inflexible way.
- ☐ Pay little attention in checking how well pt.s have understood.
- ☐ Less than half of the patients psychological morbidity is recognized



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Reasons for Patients not disclosing problems

1. Beliefs that nothing can be done.
2. Reluctant to burden the doctor.
3. Desire not to appear pathetic or ungrateful.
4. Concern that it is not legitimate to mention them.
5. Doctors blocking behavior
6. Worry that their fears about what is wrong with them will be confirmed.
7. Lack of confidentiality and trust.



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Blocking Behavior of Doctors

- Offering advice and reassurance before the main problems have been identified.
- Explaining away distress as normal.
- Attending to physical aspects alone.
- Switching the topics.
- Jollyng patients along



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What is a failed consultation or counseling

1. No rapport
2. Using medical jargon
3. Not exploring the patients agenda
4. No eliciting the actual problem
5. No contingency plan.
6. No summarization.
7. Failing to clarify
8. Not exploring in socio-cultural & economic context



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Problems & Limitations in Communication and Counseling

1. Shortage of time
2. Language barrier
3. Firm misconceptions and myths
4. Lack of awareness
5. Not ready to take responsibility for own illness.
6. Socio-cultural and economic barriers
7. Fatalistic attitude (it is God's will)



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Barriers to Communication / Counseling in Clinical Practice

1. Personal barriers
2. Lack of training
3. Undervaluing importance of communications
4. Focus only on treating diseases
5. Organization barriers
6. Lack of time
7. Work pressure
8. interruption



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When Doctors use Counseling Skills Effectively:

1. Patients problems identified more accurately.
2. Patients more satisfied with their care.
3. Patients more likely to comply with the treatment.
4. Patient's distress & vulnerability to anxiety and depression will be lessened.



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When Doctors use Consultation Skills Effectively:

- Doctors and patients wellbeing improved.
- Few clinical errors are made.
- Patients are less likely to complain.
- Reduced the likelihood of doctors being sued.



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Consultation and Counseling

- Longer duration of Consultation and Counseling
result in lesser prescription of drugs and more
patient satisfaction.



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Conclusion

- The traditional medical model of consultation and counseling does not recognize the complexity and diversity of consultation and counseling in family medicine.
- The consultation and counseling models are many. Each views the process from a slightly different prospective.
- The consultation and counseling models should match the individual needs of the patients and doctors.



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Thank You



1.

Family Medicine is best described as:

- A. The study of diseases in one organ system
- B. Care for children and pregnant women only
- C. Continuous, comprehensive care for individuals and families of all ages
- D. Emergency-only practice

Answer:



C. Continuous, comprehensive care for individuals and families of all ages



2.

What makes Family Medicine unique compared to other specialties?

- A. Focus on hospital-based cases
- B. Synthesis of physical, psychological, and social aspects
- C. Focus on surgeries
- D. Treats only chronic diseases

Answer:



B. Synthesis of physical, psychological, and social aspects



3.

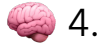
Which of the following is not a principle of family medicine?

- A. Doctor-patient-family relationship
- B. High-quality care for all ages
- C. Community-oriented practice
- D. Treating only curable diseases

Answer:



D. Treating only curable diseases



4.

Counseling primarily aims to:

- A. Make decisions for the patient
- B. Help the patient gain self-understanding and coping skills
- C. Prescribe appropriate medication
- D. Collect detailed laboratory data

Answer:



B. Help the patient gain self-understanding and coping skills



5.

A good counselor must show all of the following except:

- A. Warmth
- B. Genuineness
- C. Empathy
- D. Authoritarianism

Answer:



D. Authoritarianism



6.

Which counseling type allows the doctor to lead and direct the patient?

- A. Non-directive
- B. Reflective
- C. Directive
- D. Passive

Answer:



C. Directive



7.

Which stage of counseling involves planning and acting on solutions?

- A. Exploration
- B. New understanding
- C. Goal setting
- D. Action

Answer:



D. Action



8.

During counseling, if the doctor helps the patient see their situation from a new perspective, it's which stage?

- A. Exploration
- B. New understanding
- C. Goal setting
- D. Planning

Answer:



B. New understanding



9.

If a doctor and patient fail to reach a shared understanding of the problem, what is the likely result?

- A. Faster recovery
- B. Improved adherence
- C. Patient will not follow advice
- D. Stronger rapport

Answer:

 C. Patient will not follow advice

 10.

Which is a blocking behavior by the doctor?

- A. Listening carefully
- B. Explaining away distress as normal
- C. Clarifying the patient's fears
- D. Allowing silence for reflection

Answer:

 B. Explaining away distress as normal

 11.

Which of the following is not a reason why patients may hide their problems?

- A. Fear of bad news
- B. Desire not to burden the doctor
- C. Confidence that the doctor will listen carefully
- D. Lack of trust

Answer:

 C. Confidence that the doctor will listen carefully

 12.

Which of the following does not characterize a failed consultation?

- A. No rapport
- B. No contingency plan
- C. No summarization
- D. Checking the patient's understanding

Answer:

 D. Checking the patient's understanding

🔖 13.

A "fatalistic attitude" means:

- A. Refusing to follow treatment because of disbelief
- B. Believing fate or God controls the outcome, so nothing can be done
- C. Expecting fast recovery
- D. Trusting the doctor too much

Answer:

 B. Believing fate or God controls the outcome, so nothing can be done

🕒 14.

According to the lecture, longer consultation time results in:

- A. More drug prescriptions
- B. Less patient satisfaction
- C. Less drug use and higher satisfaction
- D. Poorer communication

Answer:

 C. Less drug use and higher satisfaction

📈 15.

When doctors use counseling skills effectively, all occur except:

- A. Patients are more satisfied
- B. Fewer clinical errors
- C. More complaints and lawsuits
- D. Less patient anxiety

Answer:

➡ C. More complaints and lawsuits

🧩 Clinical & Case-Based Questions

👩‍⚕️ 16.

A doctor interrupts a patient early to reassure them before hearing the full story. This is an example of:

- A. Active listening
- B. Blocking behavior
- C. Empathetic approach
- D. Reflective counseling

Answer:

➡ B. Blocking behavior

💬 17.

A 45-year-old woman says she feels hopeless about her illness. The doctor replies, "Don't worry, you'll be fine!" without exploring her emotions. What mistake did the doctor make?

- A. Good reassurance
- B. Over-counseling
- C. Blocking the patient's expression
- D. Using directive counseling properly

Answer:

➡ C. Blocking the patient's expression

🧠 18.

A patient says, "I don't think anything can help me; it's God's will." What type of barrier is this?

- A. Communication barrier
- B. Fatalistic attitude
- C. Language barrier
- D. Cultural bias

Answer:

 B. Fatalistic attitude

 19.

A doctor gives a complicated explanation full of medical terms, and the patient leaves confused. This consultation failed because of:

- A. Poor rapport
- B. Medical jargon
- C. Lack of empathy
- D. Overconfidence

Answer:

 B. Medical jargon

 20.

During counseling, the patient and doctor agree on goals, but the doctor fails to explain what to do if symptoms worsen. What's missing?

- A. Rapport
- B. Contingency plan
- C. Clarification
- D. Exploration

Answer:

 B. Contingency plan

 21.

A family physician holds regular follow-ups, listens to patient concerns, and involves family in decisions. Which principle does this reflect?

- A. Disease-centered care
- B. Doctor-patient-family relationship
- C. Directive approach
- D. Time-limited care

Answer:

 B. Doctor-patient-family relationship

 22.

A patient says, "I'm afraid I might have cancer." The doctor changes the topic to diet. What behavior is this?

- A. Active listening
- B. Switching topic (blocking)
- C. Empathy
- D. Clarification

Answer:

 B. Switching topic (blocking)

 23.

A physician focuses only on prescribing medication without exploring the patient's beliefs or worries. This reflects:

- A. Patient-centered care
- B. Doctor-centered care
- C. Directive counseling
- D. Reflective counseling

Answer:

➡ B. Doctor-centered care



24.

A patient feels more confident, relaxed, and motivated after a session. Which counseling stage was likely successful?

- A. Exploration
- B. New understanding
- C. Goal setting
- D. Action

Answer:

➡ B. New understanding



25.

A doctor carefully listens, summarizes the patient's main concerns, and asks, "Did I get that right?"

This demonstrates:

- A. Empathy and active listening
- B. Blocking behavior
- C. Ignoring the patient's agenda
- D. Jargon use

Answer:

➡ A. Empathy and active listening

Perfect 🙌 Let's now go deeper with Advanced MCQs (26-40) – these are exam-style, clinical reasoning & OSCE-type questions about consultation and counseling in family medicine.

 ♂ 26.

A patient says, "Doctor, I've been feeling very tired lately."

The doctor immediately orders blood tests without asking any further questions.

Which counseling skill was missed?

- A. Exploration
- B. Action
- C. Goal setting
- D. Summarization

Answer:

 A. Exploration

(The doctor skipped the exploration stage – they must first let the patient express and clarify their problem before acting.)

 27.

During a consultation, a doctor asks, "What do you think is causing your symptoms?"

This question aims to:

- A. Test the patient's knowledge
- B. Explore the patient's perspective
- C. Distract from the real cause
- D. End the session quickly

Answer:

 B. Explore the patient's perspective

(This helps understand the patient's beliefs, fears, and expectations.)

 28.

Which behavior best represents patient-centered medicine?

- A. Physician dominates the discussion
- B. Focus only on the disease and treatment

- C. Shared decision-making and respect for patient views
- D. Ignoring patient's emotions to stay objective

Answer:

➡ C. Shared decision-making and respect for patient views

🧠 29.

A patient says: "My chest hurts whenever I'm anxious."
The doctor replies: "Tell me more about what happens when you feel anxious."
This is an example of:

- A. Blocking
- B. Leading question
- C. Reflective listening
- D. Summarization

Answer:

➡ C. Reflective listening

🧑 30.

During counseling, the doctor says:
"If your symptoms get worse, come back immediately."
This statement represents which element of effective counseling?

- A. Contingency plan
- B. Exploration
- C. Rapport
- D. Directive advice

Answer:

➡ A. Contingency plan



31.

A patient starts crying during a session. The doctor silently offers tissues and waits before speaking.

This shows:

- A. Avoidance
- B. Empathy
- C. Blocking
- D. Directive behavior

Answer:



B. Empathy

(Non-verbal empathy can be more powerful than words.)



32.

A patient hesitates to share emotional problems because they feel "it's not appropriate to tell the doctor."

This is due to:

- A. Cultural misconception
- B. Lack of confidentiality and trust
- C. Fatalistic attitude
- D. Poor literacy

Answer:



B. Lack of confidentiality and trust



33.

In a 10-minute consultation, the doctor feels rushed and distracted, leading to poor communication.

This is an example of:

- A. Patient barrier
- B. Organizational barrier
- C. Doctor's lack of knowledge

D. Fatalistic attitude

Answer:

➡ B. Organizational barrier
(Time pressure and workload are system-related barriers.)



34.

A physician who practices non-directive counseling will usually:

- A. Lead and give orders
- B. Listen, reflect, and support
- C. Avoid discussing emotions
- D. Interrupt frequently

Answer:

➡ B. Listen, reflect, and support



35.

The doctor summarizes the patient's main points, checks understanding, and asks for confirmation.

This step helps to:

- A. End the consultation quickly
- B. Build mutual understanding and clarity
- C. Focus only on diagnosis
- D. Replace empathy

Answer:

➡ B. Build mutual understanding and clarity



36.

A patient says, "I think my illness is a punishment from God."

Which doctor response shows cultural sensitivity and effective counseling?

- A. "That's not true, science doesn't say that."
- B. "I understand that's how you see it; let's talk about what might help you feel better."
- C. "Ignore those thoughts and take your medicine."
- D. "That belief is wrong."

Answer:

➡ B. "I understand that's how you see it; let's talk about what might help you feel better."

(Shows empathy and respect while gently guiding the patient toward coping.)

💊 37.

Which of the following is most likely in a failed consultation?

- A. Rapport and trust built
- B. Clear summary and follow-up plan
- C. No exploration of the patient's agenda
- D. Use of simple, understandable language

Answer:

➡ C. No exploration of the patient's agenda

👩‍⚕️ 38.

A patient with chronic illness attends multiple visits over months. The doctor uses each visit to learn more and build trust gradually.

This represents:

- A. Fragmented care
- B. Family-centered continuity
- C. Single-visit management
- D. Time-limited intervention

Answer:

➡ B. Family-centered continuity

 39.

A doctor only treats the physical symptoms and ignores the patient's stress and social situation. This is an example of:

- A. Holistic approach
- B. Blocking behavior
- C. Empathetic listening
- D. Reflective counseling

Answer:

 B. Blocking behavior

 40.

A family physician who uses good consultation skills will likely have all the following outcomes except:

- A. Fewer patient complaints
- B. Lower risk of lawsuits
- C. More accurate problem identification
- D. Increased prescription rates

Answer:

 D. Increased prescription rates
